

ADMISSION NOTE - Patient synth_05 - Post-operative recovery after open cholecystectomy

PATIENT: Morgan, Arthur | MRN: synth_05 | ADMIT DATE: [Date]

CHIEF COMPLAINT: Right upper quadrant abdominal pain.

HISTORY OF PRESENT ILLNESS: This is a 68-year-old male with a past medical history of hypertension and hyperlipidemia who presents with a two-day history of progressively worsening right upper quadrant (RUQ) abdominal pain. He describes the pain as colicky, sharp, radiating to his right scapula, and rated 8/10 in severity. The pain is exacerbated by fatty food intake. He has associated nausea, multiple episodes of non-bloody, non-bilious emesis, and subjective fevers at home. He denies jaundice, dark urine, or light-colored stools. He has taken Tylenol with minimal relief.

PAST MEDICAL HISTORY: Hypertension, Hyperlipidemia.

PAST SURGICAL HISTORY: None.

MEDICATIONS ON ADMISSION: Lisinopril 20mg daily, Atorvastatin 40mg daily.

ALLERGIES: No Known Drug Allergies.

SOCIAL HISTORY: Retired construction worker. Former smoker (1/2 PPD x 40 years, quit 5 years ago). Occasional alcohol use (2-3 beers/week).

REVIEW OF SYSTEMS: As per HPI. Otherwise negative for chest pain, shortness of breath, dysuria, or changes in bowel habits.

PHYSICAL EXAMINATION:

Vitals: T 38.2 C, HR 98, BP 145/88, RR 18, SpO2 97% on room air.

General: Alert and oriented, appears uncomfortable.

Cardiovascular: Regular rate and rhythm, no murmurs.

Pulmonary: Lungs clear to auscultation bilaterally.

Abdomen: Soft, non-distended. Marked tenderness to palpation in the RUQ with a positive Murphy's sign. Normoactive bowel sounds. No rebound or guarding.

LABORATORY DATA: WBC 15.2, Hgb 14.1. AST 150, ALT 180, Alk Phos 350, Total Bili 2.1. Amylase 190, Lipase 250.

IMAGING: Abdominal ultrasound demonstrated a thickened gallbladder wall, pericholecystic fluid, and multiple gallstones, consistent with acute cholecystitis. Common bile duct was not dilated.

ASSESSMENT & PLAN:

1. Acute Calculous Cholecystitis: Given clinical presentation, physical exam, and imaging. Patient will be made NPO, started on IV fluids (Lactated Ringers) and IV Piperacillin-Tazobactam. Scheduled for open cholecystectomy in AM.
2. Hypertension: Continue home lisinopril. Monitor BP.
3. Hyperlipidemia: Continue home atorvastatin.
4. Mildly elevated pancreatic enzymes: Possibly secondary to stone passage. Will monitor clinically. Rule out pancreatitis.
5. DVT Prophylaxis: Start Enoxaparin 40mg SC daily.

PROGRESS NOTE ? Day 1 - Patient synth_05 - Post-operative recovery after open cholecystectomy

PATIENT: Morgan, Arthur | MRN: synth_05 | POD #1

SUBJECTIVE: Patient is resting comfortably. He reports his abdominal pain is much improved, now localized to the incision and rated 4/10. He is tolerating sips of water and has passed flatus. No nausea or vomiting overnight. He is using his morphine PCA for pain control.

OBJECTIVE:

Vitals: T 37.5 C, HR 85, BP 130/80, RR 16, SpO2 98% on 2L NC.

Exam: Dressing over RUQ incision is clean, dry, and intact. Abdomen is soft with hypoactive bowel sounds. Mild tenderness around the surgical site. Lungs have some diminished sounds at the bases, but he is using his incentive spirometer well.

Labs: WBC is down-trending to 11.5. LFTs improving: AST 110, ALT 135, T. Bili 1.8.

ASSESSMENT & PLAN:

68-year-old male, status post open cholecystectomy, on post-operative day 1. Recovery is proceeding as expected.

1. Post-op Pain: Continue PCA. Plan to transition to oral pain medication tomorrow.
2. Diet: Advance to clear liquids as tolerated today.
3. IV Fluids/Antibiotics: Continue D5 1/2NS at 75ml/hr. Continue IV Pip/Tazo.
4. Mobility/Pulmonary: Encourage ambulation with assistance and continued use of incentive spirometry.
5. DVT Prophylaxis: Continue Enoxaparin.
6. HTN/HLD: Continue home medications.

PROGRESS NOTE ? Day 2 - Patient synth_05 - Post-operative recovery after open cholecystectomy

PATIENT: Morgan, Arthur | MRN: synth_05 | POD #2

SUBJECTIVE: Patient states he is 'feeling a world of difference'. Incisional pain is well-controlled with oral oxycodone, rated 2-3/10. He has been ambulating in the hallways with physical therapy. He tolerated a regular diet for breakfast and lunch without any nausea. He is keen to be discharged home.

OBJECTIVE:

Vitals: T 37.1 C, HR 78, BP 128/76, RR 16, SpO2 99% on room air.

Exam: Afebrile and hemodynamically stable. Surgical incision remains clean, dry, and intact. Abdomen is soft, non-tender, with active bowel sounds. Lungs are clear to auscultation bilaterally.

Labs: WBC 8.9, Hgb 13.2. AST 80, ALT 95. A repeat bilirubin was drawn this morning to ensure continued downtrend.

ASSESSMENT & PLAN:

68-year-old male on POD#2 from open cholecystectomy, with an excellent clinical course. He meets criteria for discharge.

1. Disposition: Discharge home today.
2. Meds: Discontinue all IV medications (fluids, antibiotics, PCA). Provide prescriptions for oral pain relief.
3. Wound care: Patient and wife provided with detailed wound care instructions.
4. Follow-up: Surgical clinic in 10-14 days for staple removal. PCP follow-up arranged.
5. Pending Labs: The result for this morning's bilirubin level is awaited. Clinic will follow up with patient.

LABORATORY REPORT - Patient synth_05 - Post-operative recovery after open cholecystectomy

LABORATORY REPORT - GENERAL HOSPITAL
PATIENT: Morgan, Arthur | MRN: synth_05

--- COMPLETE BLOOD COUNT ---

Test	Admission (Day 0)	POD 1	POD 2
WBC (K/uL)	15.2 (H)	11.5 (H)	8.9
Hgb (g/dL)	14.1	13.5	13.2
Plt (K/uL)	220	210	215

--- COMPREHENSIVE METABOLIC PANEL ---

Test	Admission (Day 0)	POD 1	POD 2
Sodium (mmol/L)	138	139	140
Potassium (mmol/L)	4.1	4.3	4.2
BUN (mg/dL)	18	16	15
Creatinine (mg/dL)	0.9	0.9	0.8
AST (U/L)	150 (H)	110 (H)	80 (H)
ALT (U/L)	180 (H)	135 (H)	95 (H)
Alk Phos (U/L)	350 (H)	280 (H)	220 (H)
T. Bili (mg/dL)	2.1 (H)	1.8 (H)	PENDING

--- PANCREATIC ENZYMES ---

Test	Admission (Day 0)
Amylase (U/L)	190 (H)
Lipase (U/L)	250 (H)

MEDICATION RECORD - Patient synth_05 - Post-operative recovery after open cholecystectomy

MEDICATION ADMINISTRATION RECORD

PATIENT: Morgan, Arthur | MRN: synth_05

--- Medications on Admission ---

1. Lisinopril 20 mg PO DAILY
2. Atorvastatin 40 mg PO DAILY

--- Inpatient Medications ---

- Piperacillin-Tazobactam 3.375g IV q6h (discontinued at discharge)
- Enoxaparin 40 mg SubQ DAILY (discontinued at discharge)
- Morphine PCA (discontinued at discharge)
- Acetaminophen 1000 mg PO q6h PRN pain/fever
- Ondansetron 4 mg IV q6h PRN nausea

--- Medications on Discharge ---

1. Lisinopril 20 mg PO DAILY
2. Atorvastatin 40 mg PO DAILY
3. Omeprazole 20 mg PO DAILY
4. Oxycodone 5 mg PO q4-6h PRN pain (#30 tablets)
5. Acetaminophen 500 mg PO, 1-2 tablets q6h PRN pain

DISCHARGE ADVICE - Patient synth_05 - Post-operative recovery after open cholecystectomy

DISCHARGE INSTRUCTIONS

PATIENT: Morgan, Arthur | MRN: synth_05

DATE OF DISCHARGE: [Date]

CONDITION AT DISCHARGE: Stable, afebrile, tolerating regular diet, ambulating independently. Pain controlled on oral medication.

FINAL DIAGNOSES:

1. Acute Calculous Cholecystitis, status post open cholecystectomy
2. Acute Pancreatitis (resolved)
3. Hypertension
4. Hyperlipidemia

FOLLOW-UP:

- Please schedule an appointment with the Surgical Clinic in 10-14 days for wound check and staple removal. Call 555-1234 to schedule.
- Follow up with your Primary Care Physician, Dr. Evans, in 2-3 weeks.

WOUND CARE:

- Keep the incision clean and dry. You may shower, but gently pat the area dry afterwards. Do not soak in a bath, hot tub, or pool.
- Do not apply any lotions, powders, or ointments to the incision.
- Call the clinic or go to the emergency room for signs of infection: increasing redness, warmth, swelling, pus-like drainage, or a fever > 38.5C (101.5F).

ACTIVITY:

- No lifting anything heavier than 10 pounds (a gallon of milk) for 4-6 weeks.
- Walk several times a day to aid recovery and prevent blood clots.
- Do not drive while taking narcotic pain medication (Oxycodone).

PENDING RESULTS:

- A blood test for your liver function (Total Bilirubin) was drawn this morning and the result is still pending. The surgical clinic will call you with this result in the next 1-2 days. Please contact us if you develop new jaundice (yellowing of skin or eyes).